

SRI VENKATESHWARAA MEDICAL COLLEGE HOSPITAL & RESEARCH CENTRE- 605102

DEPARTMENT OF GENERAL SURGERY

UNIT CHIEF VI: [REDACTED]

DISCHARGE SUMMARY

NAME:	[REDACTED]	DOA: 14/12/2024
AGE/ GENDER:	[REDACTED]	DOS: 17/12/2024
IP NO:	[REDACTED]	DOD: 23/12/2024
DIAGNOSIS:	CHOLELITHIASIS	
PROCEDURE:	LAPAROSCOPIC CHOLECYSTECTOMY UNDER GA	
COMORBIDITIES	TYPE 2 DIABETES MELLITUS SYSTEMIC HYPERTENSION	
CONSULTANTS	DR. [REDACTED] (DMS & PROFESSOR) DR. [REDACTED] (ASSOCIATE PROFESSOR) DR. [REDACTED] (SR)	

BRIEF HISTORY:

C/O UPPER ABDOMINAL PAIN ON AND OFF X 4 MONTHS, INSIDIOUS IN ONSET, GRADUALLY PROGRESSIVE IN COURSE, DULL ACHING TYPE, NO AGGRAVATING AND RELIEVING FACTORS
H/O VOMITING - 1 EPISODE CONTAINS FOOD PARTICLE.

NO H/O FEVER

NO H/O CONSTIPATION/LOOSE STOOLS

NO H/O HEARTBURN/DYSPEPSIA

NO H/O HEMATEMESIS/MELENA

NO H/O SIMILAR COMPLAINTS IN THE PAST

PAST HISTORY:

K/C/O TYPE 2 DM AND SYSTEMIC HYPERTENSION X 20 YEARS ON REGULAR MEDICATION

N/K/C/O TUBERCULOSIS, BRONCHIAL ASTHMA, SEIZURE DISORDER & THYROID DISORDERS

PHYSICAL EXAMINATION: CONSCIOUS, ORIENTED, AFEBRILE.

VITALS: PR: 98 BPM BP: 140/70 MM HG TEMP: 98.6 F RR: 20 CYCLES /MIN

SYSTEMIC EXAMINATION:

CVS: S1,S2 +, NO MURMURS

RS: B/L AIR ENTRY +, NO ADDED SOUNDS

CNS: NO FOCAL NEUROLOGICAL DEFICIT.

PER ABDOMEN:

INSPECTION:

UMBILICUS IN MIDLINE

ABDOMEN DISTENDED

ALL QUADRANTS MOVES EQUALLY WITH RESPIRATION

NO VISIBLE GASTRIC/INTESTINAL PERISTALSIS

NO VISIBLE MASS

NO SINUS, DILATED VEINS

B/L HERNIAL ORIFICE FREE

PALPATION:

SOFT

NO WARMTH

TENDERNESS + OVER RIGHT HYPOCHONDRUM

NO GUARDING/RIGIDITY

NO PALPABLE MASS

NO RENAL ANGLE TENDERNESS

PERCUSSION:

TYMPANIC NOTE PRESENT

AUSCULTATION:

BOWEL SOUNDS+

INVESTIGATIONS: AS ENCLOSED**OPERATIVE PROCEDURE : LAPAROSCOPIC CHOLECYSTECTOMY :**

UNDER STERILE ASEPTIC PRECAUTION, UNDER GENERAL ANAESTHESIA WITH PATIENT IN SUPINE POSITION .PARTS PAINTED AND DRAPED. 5 PORTS - TWO 10 MM PORTS MADE AT SUPRAUMBILICAL AND EPIGASTRIC REGION AND THREE 5MM PORTS MADE AT RIGHT FLANK ALONG THE ANTERIOR AXILLARY LINE , RIGHT HYPOCHONDRUM ALONG MIDCLAVICULAR LINE AND OTHER IN LEFT HYPOCHONDRUM 2 CMS MEDIAL TO MIDCLAVICULAR LINE. ABDOMEN VISUALIZED, GALLBLADDER ADHERING TO THE SURROUNDING STRUCTURES ENCASED BY PERITONEUM AND A THICK OMENTUM AND GALLBLADDER PUSHED Laterally AND ANTERIORLY. ADHESIONS RELEASED. CALOT'S TRIANGLE SKELETONIZED. CYSTIC DUCT AND CYSTIC ARTERY VISUALIZED AND CLIP APPLIED AND DISSECTED FROM THE GALLBLADDER FOSSA INTOTO. HEMOSTATSIS ACHIEVED. GALLBLADDER EXTRACTED. PORT CLOSURE DONE. SPECIMEN SENT FOR HPE. STERILE DRESSING DONE .

COURSE IN HOSPITAL :

PATIENT WAS ADMITTED WITH THE ABOVE MENTIONED COMPLAINTS. ALL NECESSARY INVESTIGATIONS WERE DONE. PATIENT WAS DIAGNOSED TO HAVE CHOLELITHIASIS. PATIENT WAS PLANNED FOR LAPAROSCOPIC CHOLECYSTECTOMY UNDER GENERAL ANESTHESIA. GENERAL MEDICINE OPINION WAS SOUGHT IN VIEW OF GLYCEMIC CONTROL AND SYSTEMIC HYPERTENSION AND THEIR ORDERS WERE FOLLOWED. PULMOLOGY OPINION WAS SOUGHT IN VIEW OF X RAY CHANGES. CARDIOLOGY OPINION WAS OBTAINED REGARDING FITNESS FOR SURGERY. 2 UNITS PRBC TRANSFUSION WAS DONE. PAC WAS OBTAINED .LAPAROSCOPIC CHOLECYSTECTOMY WAS DONE . INTRAOPERATIVE AND POST OPERATIVE PERIODS WERE UNEVENTFUL. PATIENT WAS BEING MANAGED WITH IV FLUIDS, ANTIBIOTICS, ANALGESICS AND OTHER SUPPORTIVE MEASURES. UPPER GI ENDOSCOPY DONE ON 23/12/2024 AND FOUND TO HAVE PANGASTRITIS. PATIENT IMPROVED SYMPTOMATICALLY. HENCE PATIENT IS BEING DISCHARGED WITH FOLLOWING MEDICAL ADVICE.

TREATMENT:

- IV FLUIDS
- INJ. TAXIM 1G IV BD
- INJ. METRO 500MG IV TDS
- INJ. TRAMADOL 50 MG IM BD
- INJ. RANTAC 50 MG IV BD
- INJ. VOVERON 50 MG IM BD
- INJ. H INSULATARD 0-0-8 U S/C
- INJ. H ACTRAPID 8-8-6 U S/C
- INJ. HYDROCORT 100 MG IV OD
- TAB. METFORMIN 500 MG 1-0-1
- TAB. AMLONG 10 MG 1-0-0
- TAB. ENALAPRIL 5 MG 0-0-1
- NEB DUOLIN Q8H
- NEB FORACORT 0.5 MG Q12H
- SYP. SUCRALFATE 10ML-10ML-10ML

CONDITION AT DISCHARGE:

CONSCIOUS, ORIENTED, AFEBRILE, HYDRATION- FAIR

VITALS: PR: 68 BPM

BP: 110/60 MM HG

TEMP: AFEBRILE

RR: 18 CYCLES /MIN

SPO2: 98 % AT RA

SYSTEMIC EXAMINATION:

CVS: S1, S2 +, NO MURMURS

RS: B/L AIR ENTRY+, NO ADDED SOUNDS

PER ABDOMEN: SOFT, NON-TENDER, BOWEL SOUNDS +

CNS: NO FOCAL NEUROLOGICAL DEFICIT

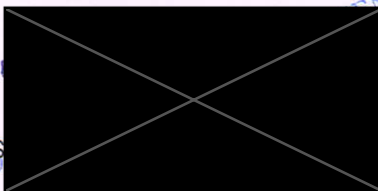
LOCAL EXAMINATION : WOUND HEALTHY , SUTURES INTACT

FOLLOW UP:

- TAB TAXIM O 200 MG 1-0-1 X5 DAYS
- [REDACTED]
- TAB. GANATON TOTAL 1-0-1 X5DAYS
- TAB.NEUROBION FORTE 1-0-0 X1 WEEK
- TAB.GLYCOMET GP 1-0-1 TO CONTINUE(30 MINS BEFORE FOOD)
- TAB.AMLONG 10MG 1-0-0 X TO CONTINUE
- TAB.ENALAPRIL 5MG 0-0-1 TO CONTINUE
- SYP.SUCRAFIL 10ML-10ML-10ML
- SYP. DIGIRAF 10ML-0-10ML
- MDI FORACORT 200MG 2PUFFS BD VIA SPACER
- [REDACTED]
- REVIEW AFTER 1 WEEK IN GENERAL SURGERY VI OPD ON 28/12/2024
- REVIEW AFTER 2 WEEK IN GENERAL MEDICINE OPD WITH FBS AND PPBS REPORT
- REVIEW AFTER 2 WEEKS IN PULMONARY MEDICINE OPD WITH MDI INHALER

JR

Signature
SR/ASS



UNIT CHIEF